

Parkinson's Disease (DC 8004) Minimum evaluation 30%; Use suggested diagnostic codes below when evaluating manifestations of Parkinson's disease beyond the minimum. Choose diagnostic code(s) that most nearly approximates the disability(ies) shown.

Manifestation	Suggested DC	Comments
Stooped posture	8211	Separate evaluations warranted for left side and right side involvement; bilateral factor does not apply
Balance impairment	6204	For use when balance impairment is present without lower extremity impairment. Otherwise, consider balance impairment as part of extremity evaluation [shown below]
Bradykinesia or slowed motion		Consider as part of extremity evaluations
Loss of automatic movements (face masking, drooling, etc)	8207	Separate evaluations warranted for left side and right-side involvement; bilateral factor does not apply
Speech changes	• 8210 bilaterally, • 6516, or • 6519	Any of these three DCs can be used. Assign whichever results in the highest evaluation. Bear in mind, cranial nerves are rated bilaterally.
Upper extremity • tremors • muscle rigidity • stiffness, and/or • bradykinesia or slowed motion	• 8514 for impairment without loss of use • 5125 for LOU of one hand • 5109 for LOU of both hands	Loss of use is defined in 38 CFR 3.350(a)(2)
Lower extremity • tremors • muscle rigidity • stiffness, and/or • bradykinesia or slowed motion	• 8520 for impairment without LOU • 5167 for LOU of one foot • 5110 for LOU of both feet	
Depression	9434	
Cognitive impairment or Dementia	9326	
Loss of sense of smell	6275	
Sleep disturbance	if manifested as a • respiratory disability, 6847 • insomnia, 9410	If another mental disorders diagnosis is also present along with insomnia, evaluate insomnia along with the other mental disorder diagnoses. For more information on evaluating insomnia, see M21-1 III.iv.4.O.1.l.
Difficulty chewing/swallowing	• 7203, or • 8205	
Urinary problems	7542	
Constipation	7319	
Sexual dysfunction/ erectile Dysfunction	7522	When associated to Parkinson's Disease diagnosis, grant SMC K, see M21-1 III.iv.4.I.3.b.

Multiple Sclerosis - (DC 8018) Minimum evaluation 30%; Use suggested diagnostic codes provided below when evaluating manifestations of MS beyond the minimum. Choose diagnostic code(s) that most nearly approximates the disability(ies) shown.

Manifestation	Suggested DC	Comments
Speech difficulties, including hoarseness	6519 6516	
Paralysis of soft pallet with swallowing difficulty and speech impairment	6521	
Swallowing difficulties	7203	
Respiratory conditions		If present, respiratory DBQ should be of record. Based on symptoms shown, select the most advantageous DC from 38 CFR 4.97 Schedule of ratings - Respiratory system
Sleep disturbances	6847 - for sleep apnea or sleep apnea-like symptoms 9410 - for insomnia	If another mental disorders diagnosis is also present along with insomnia, evaluate insomnia along with the other mental disorders diagnosis.
Loss of sphincter control with or without leakage	7332	
Chronic constipation	7319	
Voiding dysfunction	7542 for leakage, frequency, or obstructed voiding	Only a single evaluation is warranted even if more than one dysfunction / symptom type is present. May be presented as symptoms only, versus an official diagnosis.
Recurrent urinary tract infections	7527	
Erectile dysfunction	7522	When associated to Multiple Sclerosis (MS) diagnosis, grant SMC K, see M21-1 III.iv.4.I.3.b.
Visual disturbances	6090 - Diplopia or internuclear ophthalmoplegia 6066 - decreased visual acuity or blurring of vision 6081 - visual scotoma 6016 - Nystagmus 6026 - Optic neuritis	For visual impairment, a single evaluation is warranted either under the general rating formula, OR based on visual impairment criteria (consisting of visual acuity, visual field, and impairment of muscle function)
Upper extremity(ies)	8514 for impairment without loss of use (LOU); separate evaluations warranted for bilateral involvement 5125 for LOU of one hand 5109 for LOU of both hands	Synthesize findings of neurologic exam (gait strength, deep tendon reflexes, sensation, muscle atrophy / weakness) and remaining effective function to assess level of impairment for the upper extremity, to include whether loss of use is present. LOU is defined in 38 CFR 3.350(a)(2)

Multiple Sclerosis - (DC 8018) Minimum evaluation 30%; Use suggested diagnostic codes provided below when evaluating manifestations of MS beyond the minimum. Choose diagnostic code(s) that most nearly approximates the disability(ies) shown.

Lower extremity(ies)	• 8520 for impairment without LOU; separate evaluations warranted for bilateral involvement • 5167 for LOU of one foot • 5110 for LOU of both feet	Synthesize findings of neurologic exam (gait, strength, deep tendon reflexes, sensation, muscle atrophy / weakness) and remaining effective function to assess level of impairment for the lower extremity, to include whether loss of use is present. LOU is defined in 38 CFR 3.350(a)(2)
Depression	9434	
Cognitive impairment or Dementia	9326	

Amyotrophic Lateral Sclerosis - (DC 8017) Minimum evaluation 100%; Use suggested diagnostic codes provided below when evaluating manifestations of ALS beyond the minimum. Choose diagnostic code(s) that most nearly approximates the disability(ies) shown.

Manifestation	Suggested DC	Comments
Speech difficulties, including hoarseness	6519, 6516, or 8210 (assign two evaluations; left and right tenth cranial nerve)	Select evaluation criteria that provides the most advantageous evaluation.
Paralysis of soft pallet with swallowing difficulty and speech impairment	6521	
Swallowing difficulties	7203	
Respiratory conditions (including requirement for tracheostomy)	6520	If present, PFT results should be of record.
Sleep apnea or sleep apnea-like condition	6847	
Loss of sphincter control with or without leakage	7332	
Chronic constipation	7319	
Voiding dysfunction	7542 for leakage, frequency, or obstructed voiding	Only a single evaluation is warranted, even if more than one dysfunction / symptom type is present. May be presented as symptoms only, versus an official diagnosis.
Recurrent urinary tract infections	7527	
Erectile dysfunction	7522	When associated to (ALS) diagnosis, grant SMC K, see M21-1 III.iv.4.I.3.b.
Upper extremity(ies)	8513 for impairment without loss of use (LOU); separate evaluations warranted for bilateral involvement 5125 for LOU of one hand 5109 for LOU of both hands	Synthesize findings of neurologic exam (gait, strength, reflexes, muscle weakness / atrophy) and remaining effective function to assess level of impairment for the upper extremity, to include whether loss of use is present. LOU is defined in 38 CFR 3.350(a)(2)
Lower extremity(ies)	8520 for impairment without LOU; separate evaluations warranted for bilateral involvement 5167 for LOU of one foot 5110 for LOU of both feet	Synthesize findings of neurologic exam (gait, strength, reflexes, sensation, muscle atrophy / weakness) and remaining effective function to assess level of impairment for the lower extremity, to include whether loss of use is present. LOU is defined in 38 CFR 3.350(a)(2)
Depression	9434	
Cognitive impairment or Dementia	9326	